

Discharge Summary USE BLACK INK ONLY

Patient Details Surname Clarke Forename Emma M / F/ (Female) Date of Birth 14/03/1998 NHS/ Hosp No. 943 762 5810 Address 14 Birchwood Close, Reading, RG6 1PL Tel No. 0118 496 2233		Admission and GP Details Discharging Consultant Mr J. Whitmore Discharging Speciality/ Department General Surgery Method of Admission Emergency - via A&E Date of Admission 21/07/2026 Date of Discharge 23/07/2026 G.P. Details Dr S. Ahmed, Thameside Medical Practice, Reading	
Diagnosis at Discharge Acute cholecystitis with cholelithiasis.		Operations and Procedures Laparoscopic cholecystectomy (21/07/2026), uncomplicated.	
Reason for Admission and Presenting Complaint(s) 2-day history of severe constant RUQ pain radiating to the right shoulder tip, with fever, nausea and vomiting. Positive Murphy's sign on examination, no jaundice.			
Clinical Narrative Previously fit and well, presented with biliary colic progressing to acute cholecystitis. Bloods showed raised WCC/CRP with normal LFTs and bilirubin. USS confirmed cholelithiasis with gallbladder wall thickening and pericholecystic fluid. Commenced on IV co-amoxiclav and analgesia, underwent laparoscopic cholecystectomy on day of admission without complication. Post-op recovery uncomplicated - tolerating diet, mobilising independently, pain controlled on oral analgesia, wounds clean and dry.			
Relevant Investigations and Results WCC 14.2, CRP 88, LFTs/bilirubin normal. USS: cholelithiasis, GB wall thickening, pericholecystic fluid.			
Discharge Destination Home, lives with partner. Fully independent with ADLs.			
Relevant legal Information (e.g. was an independent Mental Capacity Act Advocate required) No safeguarding or Mental Capacity Act concerns. Patient has full capacity.			
Information given to patient and/or authorised representative (including e.g. see GP in 2 weeks) Advised on wound care, signs of infection and return precautions. Counselling on activity/lifting restrictions and driving. Told to return to A&E if fever, worsening pain, jaundice or wound discharge/redness.			
Physical Ability & Cognitive Function :		On Admission	At Discharge
Physical	Independent	Independent, mobile	
Cognitive	Normal	Normal	
Other	N/A	N/A	
Advice, recommendations and future plans (including results awaited and outstanding investigations) G.P. Actions (Date) Suture/glue check at day 5 if any concerns; no bloods required. For GP: review in 2 weeks if concerns; no further imaging needed; stop analgesia as pain settles. Patient counselled on post-op recovery, wound care and activity/driving restrictions (see above) and safety-netted to return to A&E for fever, worsening pain, jaundice or wound problems. Patient information leaflet on laparoscopic cholecystectomy and gallstone disease provided. Strategies for potential problems Safety-netting given; patient has the surgical assessment unit contact number.			

